Fax Cover Sheet

To: +1-702-726-9551
From: Ortho Las Vegas
From Number: 888-593-7092
Date: 04/15/2026
Page Count: 5

Ortho Las Vegas

Ortho Las Vegas

Appointment: 04/29/2026 11:00 AM

Provider: Bendicion, Jake PAC

Patient Information

Name:	Karen Halaby	**Home Phone:**	
D.O.B:	02/25/1954	**Work Phone:**	
Sex:	Female	**Mobile Phone:**	(805) 501-9570
SSN:		**Email:**	karenhalaby@gmail.com
MRN:	MM0000005999	**Preferred Contact Method:**	Phone
PMS ID:	114340PAT000006009	**Language:**	Declined to Specify
Marital Status:	Unspecified	**Emergency Contact:**	
Race:	Declined to Specify	**Emergency Contact Phone:**	
Ethnicity:	Declined to Specify	**Employer:**	
Address:	420 Turtle Mesa Avenue Henderson, NV 89011	**Primary Care Provider:**	
		Referring Providers:	

Primary Insurance Information

Carrier:	Medicare of NV	**Address:**	PO Box 6727 Fargo, ND 581086727
Policy #:	3RP2D71FW27		
Group ID/Name:		**Phone Number:**	

Secondary Insurance Information

Carrier:	Humana Health Plan PPO	**Address:**	
Policy #:	H63669536		
Group ID/Name:	9A761	**Phone Number:**	

Pharmacy Information

Name:	CVS/pharmacy #0121		
Phone Number:	7025585101	**Address:**	1402 E LAKE MEADE PKWY HENDERSON, NV 89015
Fax Number:	7025585680		

ORTHO

Halaby, Karen

Visit Note - March 2, 2026

PMS ID	Sex	DOB	Phone	MRN
114340PAT000006009	Female	02/25/1954	(805) 501-9570	MM0000005999

Alerts
No allergy to shellfish/iodine, no defibrillator, no blood thinners, no allergy to latex, no allergy to echeshin, no pacemaker, and no rheumatoid arthritis.

Allergies
No known drug allergies

Medications
escitalopram oxalate oral
Zepbound 10 mg/0.5 mL Subcutaneous - solution
levothyroxine

Medical History
Anxiety disorder

Musculoskeletal History

Musculoskeletal Family History
Diabetes

Family History
Reviewed October 20, 2025.
Other: Her brother in law.

Social History
Occupation: Retired, Use to be in Sales.
EtOH none
Smoking status - Never smoker

Tobacco Use
Does not use vaping products
Does not use smokeless tobacco
Tobacco Product: None

Chief Complaints:
1. right knee follow up

HPI: This is a 72 year old female who:
1. returns for a follow up regarding the right knee. Patient states that her symptoms have unfortunately returned despite two steroid injections, first in November 2025 and her second in January 2026. She states that now, she is requiring assistance going from a sitting to standing position secondary to her pain. She denies any balance issues. She also feels she is losing range of motion over time. She has read about a jiffy knee procedure and inquires if this is an option today.

Exam:

Exam

Exam findings notable for the following:
Exam: The patient's skin is warm and dry, with no erythema or ecchymosis; motor and sensory function are intact, and the patient has 0-120 degrees active range of motion about the knee without pain. There is medial joint-line tenderness. There are no bony deformities. Distal pulses intact.
McMurray's Maneuver: Positive
Varus Stress Test: Negative
Valgus Stress Test: Negative
Lachman's Maneuver: Negative

X-rays: 4 views of the right knee were taken in the office on 10/29/2025. X-rays demonstrate mild osteoarthritis in the medial compartment.

Imaging: MRI right knee done at Pueblo Medical Imaging on 11/4/2025 demonstrates moderate/severe tricompartmental osteoarthritis worst in the medial compartment with bone-on-bone phenomena. Small joint effusion. Medial extrusion of the body of the medial meniscus due to osteoarthritis and joint space loss. No meniscal tear identified. Mucoid degeneration of the ACL. Patchy bone marrow edema within the distal femur and proximal tibia which is presumably degeneration in nature.

ULTRASOUND: Limited ultrasound right knee done in the office on 11/12/2025. Findings demonstrate moderate joint effusion.

Data Reviewed:
1 Independent interpretation of a test performed by another physician/other qualified health care professional (not separately reported) (MRI Interpretation Knee (MRI Knee previously ordered by this physician/extender))

Tests

MRI Interpretation Knee

MRI: right MRI Knee previously ordered by this physician/extender

William Ralston McGee, DO (Primary Provider) (Bill Under)
(702) 485-5885 Work
(888) 593-7092 Fax

Ortho Las Vegas
9159 W Flamingo Road
Suite 100
Las Vegas, NV 89147-6454

Date: 11/04/2025

MRI of the right knee was reviewed, demonstrating the following findings: moderate/severe tricompartmental osteoarthritis worst in the medial compartment with bone-on-bone phenomena. Small joint effusion. Medial extrusion of the body of the medial meniscus due to osteoarthritis and joint space loss. No meniscal tear identified. Mucoid degeneration of the ACL. Patchy bone marrow edema within the distal femur and proximal tibia which is presumably degeneration in nature.

Impression/Plan:

Right knee osteoarthritis. X-rays from 10/2025 were reviewed with the patient in detail. I explained that she is a candidate for a unicompartmental knee arthroplasty given there are only degenerative changes in her medial femorotibial compartment. We discussed the risks and benefits regarding a mako robot-assisted unicompartmental knee arthroplasty. At this point, the patient has failed conservative treatment for her knee osteoarthritis and would like to go forward with unicompartmental mako robot-assisted knee arthroplasty. We will have her evaluated medically and she will undergo risk stratification and lab work prior to surgery. She will also complete an education course related to total joint surgery prior to the procedure. I will also order a CT scan of the right knee for fitting of UKA implants. Once these all have been completed we will plan for a right unicompartmental mako robot-assisted knee arthroplasty.

I discussed the risks, alternatives, and benefits to the procedure with the patient. The risks of surgery include, but are not limited to bleeding, infection, injury to nerves or blood vessels, DVT, PE, and other risks associated with anesthesia including death. The patient understands these risks and wants to move forward with the surgery. We will schedule this surgery for the patient at their earliest convenience.

1. Knee Osteoarthritis, Right
   Problem Addressed: Stable chronic illness
   Unilateral primary osteoarthritis, right knee (M17.11)
   located on the right knee.

   Plan: Counseling - Orthobiologics.
   Orthobiologics refers to products such as tissue, cells, and molecules made by those cells called growth factors that may aid in tissue healing and regeneration.

   Autologous platelet rich plasma (PRP) is the component of blood that has been prepared to maximize the concentration of platelets and their associated growth factors. The PRP can be introduced into the painful joint or soft tissue to wake up the signaling between these molecules necessary for healing. Autologous means that your own cells are used, eliminating much of the concern regarding disease transmission or allergic reaction. PRP can be obtained from a standard blood draw. There is evidence to support the potential benefits of PRP for some tendon and joint disorders.

   Some growth factors are used in orthopedic surgery to aid the healing of bone and soft tissue structures.

   Stem cells are cells that have the potential to turn into different types, or lineages, of cells, such as the cells that make up muscle, tendon, cartilage, and bone. Stem cells produce and release molecules called growth factors that participate in communication between cells. The richest source of stem cells in the body is the bone marrow. There are basic science studies that support the ability of stem cells to help with healing and repair. There is ongoing research to establish the efficacy of stem cell delivery at the site of musculoskeletal injury.

   PRP therapy starts with drawing your blood and placing the whole blood volume in a centrifuge, which serves to separate the desired PRP cells into a richly concentrated layer. You will then be positioned to allow access of the joint or area to be injected. After sterile preparation of the area, the PRP is delivered by injection. Ultrasound is sometimes used to target the exact location for treatment. PRP can also be administered at the time of surgery. PRP can be planned as a single injection or a series of injections.

   Plan: Counseling - Knee DJD.
   Musculoskeletal care: I advised the patient that most patients with degenerative joint disease of the knee can achieve some measure of pain relief from nonsurgical options like medications, lifestyle modification, exercise and physical therapy, and use of supportive devices or change in footwear. The importance of maintaining an ideal body weight or losing weight to reduce stress on the knee joint is a cornestone of nonoperative management.
   Expectations: I counseled the patient about the natural history of degenerative joint disease of the knee which typically has exacerbations and remissions. Many people report that changes in the weather also affect the degree of pain from arthritis. The knee joint does not heal on its own when there is arthritis. With time, the symptoms of arthritis may get worse.For some,

conservative management may not be not satisfactory. These individuals should consider surgical options.
Contact Office if your knee pain worsens, or if the knee becomes warm or accompanied by a fever.

Plan: Order Surgery: Knee Arthroplasty.
SURGERY INFORMATION:

Provider: William Ralston McGee, DO
Priority: normal

Estimated Length of Procedure: 90 minutes

Diagnosis: Knee Osteoarthritis, Right - M17.11
CPT Codes:
Medial Unicondylar Knee Arthroplasty (27446)

Surgical Site: right knee

Position: Supine

Contraindications: Verified that patient has no active infection of the knee joint, no systemic bacteremia, no skin infections, no neuropathic arthritis, no progressive neurological disease, and no other severe medical conditions that outweigh benefits of TKA.

Risks and benefits: I had a lengthy conversation with the patient/family regarding the risks of surgery, which include but are not limited to infection, persistent pain, blood clots, pulmonary embolus, need for repeat surgery, cardiac events, fat embolus, swelling, stroke and death; the benefits of surgery; and the alternatives to surgery which include second opinions, delaying surgery, further conservative care, or alternative medicine. They have had ample opportunity to have all of their questions answered by the medical team and are encouraged to contact us with any questions, changes or concerns that arise prior to surgery.

Staff:

William Ralston McGee, DO (Primary Provider) (Bill Under)

Ryan Chang (scribe)

Kailey Plumlee

Ryan Chang scribing for, and in the presence of, William McGee, DO (Roddy).

I, William McGee, DO (Roddy), personally performed the services described in the documentation as scribed by Ryan Chang in my presence, and I confirm the documentation is both accurate and complete.

Electronically Signed By: William Ralston McGee, DO, 03/04/2026 10:35 PM PST

Active

6765 W Charleston Blvd Ste 170
Las Vegas, NV 89146
Office: (775) 471 - 9815
Fax: (702) 726 - 9551
Email: admin@activehomehealthcare.net

Home Health Referral

Gaby Fuentes | Account Executive
Cell: (702) 957 - 3472
Email: Gaby@activehomehealtcare.net

Patient Name: Karen Halaby

DOB: 02.25.1954

skip if demographic sheet is attached

Address: 420 Turtle Mesa Avenue Henderson, NV 89011

Tel #: (805) 501-9570

Emergency Contact #:

1st Insurance & Policy #: medicare - 3RP2D71FW27

2nd Insurance & Policy #: humana - H63669536

Medical Condition related to Home Health

- ☐ CVA
- ☐ AMI
- ☐ Diabetes
- ☐ COPD
- ☐ Cardiac Surgery
- ☐ ESRD
- ☐ Heart Failure
- ☒ Joint Replacement
- ☐ HTN
- ☐ Heart Failure
- ☐ PNA

Other, please specify:

a. Physical Therapy
b.
c.
d.
e.
f.
g.
h.
i.
j.

SX: 04.14.26 Right Total Knee Arthroplasty

Homebound due to (i.e inability to leave home, leaving home requires considerable and taxing effort and is medically contraindicated or requires assistance/ DME):

a.
b.
c.
d.
e.
f.

Clinical findings or services required ins support to patient's home health eligibility:

☐ Skilled Nursing

- __ Health education on disease process
- __ Medication reconciliation/management
- __ DM management
- __ Catheter/Drain care
- __ Post operative care
- __ Wound Care and pt/family training
- __ Ostomy care and pt/family training
- __ Central line care
- __ O2 Therapy/Trache management
- __ Others:

☒ Physcial Therapy / ☐ Occupational Therapy

- __ Gait/balance/transfer training & optimization
- __ Home safety /fall precaution
- __ Passive and/or Active ROM Exercises
- __ Post op exercises
- __ Education for safe use of assistive device
- __ Set up home exercise program
- __ Therapeutic exercise for strengthening & endurance
- __ Others:

☐ Speech Language Pathology Skilled evaluation/treatment:

- __ Aphasia
- __ Apraxia
- __ Dysarthria
- __ Dysphagia

☐ Medical Social Worker

- __ Psychosocial assessment, dentification or assistance in application for community government resources
- __ Others:

Ordering Provider: William McGee

Tel #: 702-885-9844

Signature: WM

Date: 04.15.2026

Address: 9159 W Flamingo Rd #100 Las Vegas, NV 89147